MIDTERMS: NCMB 419 (NLM) - WEEK #7: RECORDS MANAGEMENT

CONCEPT OF RECORDS MANAGEMENT

Definition of Records Management

— **Records management (RM)** is the supervision and administration of digital or paper records, regardless of format.

— It involves the systematic and effective control of:

- Paper records
- Electronic records
- Creation
- Receipt
- Maintenance
- Use
- Disposal

— The goal is to ensure records are:

- Accurate
- Reliable
- Secure
- Quickly retrievable
- Retained only for the necessary period

What is a Record?

— A **record** is a permanent written communication documenting information relevant to a client's healthcare management.

— A healthcare record may serve as a:

- Clinical document
- Scientific document
- Administrative document
- Legal document

Term	Meaning
Information	Data, ideas, thoughts, or memories regardless of medium
Document	Recorded information or object that can be treated as an individual unit
Record	Information created, received, and maintained as evidence
Archive	Records selected for permanent preservation because of their value

Importance Of Records Management

— Records are important because they:

1. Provide evidence of actions and decisions
2. Support accountability and transparency
3. Ensure compliance with legal/regulatory requirements
4. Protect the interests of patients, staff, and stakeholders
5. Help address complaints and legal proceedings
6. Support patient choice and control over treatment
7. Support daily healthcare delivery
8. Support evidence-based practice
9. Assist clinical and nursing audits
10. Support administrative and managerial decision-making
11. Support research and improvement of clinical effectiveness

Benefits Of Records Management

— Proper records management:

- Saves time
- Saves space
- Saves money
- Improves efficiency
- Improves legal/regulatory compliance
- Improves quality and reliability of information
- Increases security of confidential records
- Supports continuity of operations
- Supports risk management
- Allows timely retrieval
- Ensures appropriate disposal
- Protects records with long-term/historical value

CLASSIFICATION OF RECORDS

1. ACTIVE RECORD

— A record that is regularly referenced or currently required for use.

2. INACTIVE RECORD

— A record that is still needed by the organization but is no longer required for current operations.

3. ELECTRONIC RECORD

— A record that is recorded or formatted in a manner that requires a computer to process.

TYPES/SAMPLES OF RECORDS

A. Administrative Records

— *Examples:*

- Grants/contracts
- Bid documents
- Facility blueprints
- Consent forms
- Equipment inventory reports
- Meeting minutes
- Payroll folders
- Purchase/lease/rental contracts

B. Nursing Office and Unit Records

— *May include:*

- Organogram
- Job descriptions
- Procedure manuals
- Personnel records
- Patient records
- Leave records
- Duty rosters
- Meeting minutes
- Budget records
- Circular logbooks

C. Individual Staff Records

Contains employment-related information such as:

- Staff information
- Sickness
- Absences
- Training records

PRINCIPLES OF RECORD WRITING

— *A nurse should ensure that records are:*

1. **Clear** – Use understandable and appropriate language.
2. **Factual** – Documentation should be based on:
   - Observation
   - Conversation
   - Action
3. **Relevant** – Record only information important to patient care.
4. **Neat and Complete** – Avoid incomplete or confusing documentation.
5. **Uniform** – Use consistent documentation practices.
6. **Immediate/Timely** – Records should be written immediately after an interview or relevant event.
7. **Confidential** – Records must be protected from unauthorized access.

DATA PRIVACY PROTOCOLS

A. Data Privacy Act of 2012 — RA 10173

— The Data Privacy Act of 2012 (RA 10173) protects personal information and establishes principles for the proper processing and protection of personal data.

— For Nurses, remember:

- Maintain patient confidentiality.
- Access records only when authorized and necessary.
- Do not disclose patient information to unauthorized individuals.
- Protect paper and electronic records.
- Do not share passwords.
- Prevent unauthorized copying, photographing, or transmission of records.
- Follow institutional policies regarding release of records.

Three Important Data Privacy Principles

➢ **Transparency**

— The individual should know how and why information is processed.

➢ **Legitimate Purpose**

— Information should be processed for a lawful and legitimate purpose.

➢ **Proportionality**

— Only information necessary for the intended purpose should be collected/processed.

➢ SENTINEL EVENTS

— A sentinel event is a serious unexpected patient-safety event resulting in death, serious harm, or significant risk of serious harm.

— *Examples*

- Wrong-site surgery
- Wrong-patient procedure
- Serious medication-related event
- Patient suicide
- Serious preventable injury

— *Nurse's Priority*

1. Protect the patient
2. Notify appropriate personnel
3. Assess and provide care
4. Document patient condition and care
5. Complete required event/incident report

➢ ANECDOTAL RECORD

— A brief record describing a specific observation, behavior, or significant event.

— Characteristics

- Objective
- Specific
- Factual
- Based on direct observation

— Example

- Incorrect: "Patient is rude and difficult.“
- Correct: "Patient refused medication and stated, 'I don't want to take it.'"

➢ INCIDENT REPORT

— A report documenting an unusual, unexpected, or potentially harmful event.

— *Examples*

- Patient fall
- Medication error
- Needle-stick injury
- Patient injury
- Equipment malfunction

— *Proper Nursing Action*

- Report promptly.
- State objective facts.
- Include who, what, when, where, and how.
- Avoid blame.
- Avoid personal opinions.

➢ KARDEX

— A quick-reference nursing record containing essential information needed for current patient care.

KARDEX

NAME:	Mr. Leurdo V. Cortes		AGE:	73	SEX:	M	HOSPITAL NO.: 2021-10002
ADDRESS:	Cabasaan, Vigan City, Ilocos Sur.		CLASSIFICATION:		Private		WEIGHT: 81kg
ADMITTING PHYSICIAN:	Dr. Ivan D. Magtira		DATE/TIME ADMITTED:		4/5/2021		BLOOD TYPE: O +
ATTENDING PHYSICIAN:	Dr. Ivan D. Magtira/ Dr. Marlina A. Flores (CP clearance) / Dr. Leo Nunez (anesthesiologist)						
COMPLAINT:	Increasing size of a lump over left hip and has been causing pain and discomfort for over the past two weeks. Skin around the lump is red and swollen; 38.3°C temperature.						
IMPRESSION DIAGNOSIS:	Left hip Abscess, to rule out DVT vs Reoccurring left Hip Osteomyelitis						
SURGERY DONE:	~~Needle aspiration(4/5)~~, surgical incision and drainage		DATE/TIME OF SURGERY:				

MENTAL STATUS:	ACTIVITIES:	DIET:	TUBES:	Special Info:
☑ Conscious	☑ Ambulant	☐ NPO	☐ Foley Catheter	☐ Weigh Daily
☐ Drowsy	☐ Dangle and sit up	☐ DAT	☐ Thoracic tube	☑ BP q shif
☐ Stupor	☐ Bedrest with BRP	☐ Soft	☐ NGT	☐ Neuro V/S q1
☐ Unconscious	☐ CBR w/o BRP	☐ Clear liquids	☐ CVP	☐ Abdominal girth
☐ Comatose	Others:	☐ Gen. liquids	Others:	Others:
		Others: LSLF Diet; NPO post-midnight	PICC line at left brachial vein.	TPR q shift and record
				Keep PICC area/line clean

— May Include:

- Patient identification
- Diagnosis
- Diet
- Medications
- Treatments
- IV therapy
- Activity
- Allergies
- Precautions

- Nursing care

Date Ordered	Medication	Date Ordered	IV Fluids/Blood Transfusion	Date and Time Discontinued
4/5/2021	Start Fondaparinux 2.5 mg SQ OD	4/5/2021	PNSS 1L x 12h 27-28gtts/min	
4/5/2021	Give Acetaminophen 325-650 mg tab q4hr PRN for Mild to Moderate Pain			
4/5/2021	Start Docusate sodium 300mg cap PO OD			
4/5/2021	Start Linezolid 600mg/300mL IV OD, ANST ( )			

Date Ordered	Medical Treatment/ Laboratories/Diagnostics	Date Done
4/5/2021	For Needle Aspiration of Abscess	4/5/2021
4/5/2021	CBC	4/5/2021
4/5/2021	FBS, Lipid Profile	
4/5/2021	BUN, Crea	
4/5/2021	SGPT, SGOT	
4/5/2021	CXR-PA	
4/5/2021	12 Lead ECG	4/5/2021
4/5/2021		

IMPORTANCE OF RECORDS TO DIFFERENT PEOPLE

A. For the Patient/Family

— Records:
- Serve as the patient's health history
- Support continuity of care
- Provide evidence in legal issues
- Support research and teaching

B. For the Doctor

— Records:
- Guide diagnosis and treatment
- Show progress
- Support follow-up and evaluation
- Provide legal protection
- Support teaching and research

C. For the Nurse

— Records:
- Document nursing services rendered
- Guide planning and evaluation
- Support professional growth
- Facilitate communication with other healthcare workers
- Indicate future plans

D. For Authorities/Administration

— Records provide:
- Statistical information
- Administrative control
- Future reference
- Quality evaluation
- Staff evaluation
- Legal evidence
- Justification of expenditures

Nurse's Responsibility In Safekeeping Records

— *Nurses should:*
— Keep records under safe custody
— Ensure no individual sheet is improperly separated
— Prevent unauthorized patient/visitor access
— Prevent strangers from reading records
— Avoid releasing records to legal advisors without proper written administrative permission
— Handle records carefully
— Ensure proper patient identification
— Never send records outside the hospital without required written administrative permission

Nursing Administrator's Responsibility

— The nursing administrator is responsible for:
- Protection of records from loss
- Safeguarding records
- Completeness
- Nurse notes
- Admission records
- Scientific value of nurse notes
- Documentation of orders carried out

201 FILE / INDIVIDUAL STAFF RECORD

— A separate personnel record is maintained for staff.
— May Include:
- Personal/employment information
- Sickness records
- Absences
- Training records
- Other employment documentation

RECORDS LIFECYCLE

— *Create/Receive → Active → Inactive → Disposition*

1. CREATE/RECEIVE

— The record is:
- Created internally OR received externally
- Made complete and accurate
- Given appropriate security classification
- Distinguished from non-record/working copies
- Properly classified/filed

2. ACTIVE PHASE

— Records are:
- Frequently used
- Shared among authorized colleagues
- Retrieved for daily operations
- Properly classified
- Maintained in the filing system
- Protected from alteration

— The record's integrity and usability must be maintained.

3. INACTIVE PHASE

— Records are no longer needed for daily office operations but are still retained.
— Actions:
- Identify records no longer needed in primary storage
- Organize and list them
- Transfer them to an appropriate records center/storage
- Retrieve them when necessary

4. DISPOSITION PHASE

— Determine whether records should be:
- Permanently Retained Or Disposed/Destroyed

— For records due for destruction:
- Identify
- List
- Obtain necessary approvals
- Use an appropriate/environmentally responsible destruction process

MIDTERMS: NCMB 419 (NLM) - WEEK #8: INTRA-DISCIPLINARY, INTERDISCIPLINARY, MULTIDISCIPLINARY & WORK COLLABORATION

COLLABORATION

— Means "work together."
— Involves two or more individuals or disciplines
— Includes:
- Communication
- Information sharing
- Coordination
- Cooperation
- Problem-solving
- Negotiation

— Combines different perspectives and expertise.
— Requires a common goal.
— Involves shared authority and responsibility.
— Effective collaboration produces a "win-win-win" relationship.

TYPES OF TEAMWORK AND COLLABORATION

1. INTRA-DISCIPLINARY

— Collaboration within the same discipline/profession.
— *Examples*
- Nurse + Nurse
- Nurse Manager + Staff Nurse
- Nursing Faculty + Clinical Instructor

2. INTERDISCIPLINARY

— Combines two or more disciplines into an integrated plan.
— Professionals share expertise in:
- Practice
- Research
- Education
- Theory

3. TRANSDISCIPLINARY

— Multiple disciplines share knowledge and skills across traditional professional boundaries.
— Focuses on accomplishing a common task or goal.

4. INTERPROFESSIONAL COLLABORATION

— Two or more professional disciplines work together intentionally.
— Based on:
- Mutual respect
- Commitment
- Shared responsibility
- Common goal

CONCEPT OF TEAM COLLABORATION

1. HOSPITAL SETTING

Multidisciplinary/Interdisciplinary Team

— Different healthcare professionals contribute their:
- Knowledge
- Skills
- Expertise
- Services

— toward achieving safe and quality patient care.
— *Examples:*
- Nurse
- Physician
- Pharmacist
- Dietitian
- Radiologist
- Medical technologist
- Academic/educator

Hospital Collaboration Goals
- Safe patient care
- Quality outcomes
- Effective communication
- Coordination of services
- Continuity of care
- Efficient use of resources

2. COMMUNITY SETTING

Important forms of collaboration:

A. Intra-agency

— Collaboration within the same agency/health unit

B. Inter-agency

— Collaboration between different agencies

C. Multidisciplinary

— Different professional disciplines working toward community health goals

D. Sectoral

— Collaboration among different sectors involved in healthcare.

Examples include:
- Nurses
- Physicians
- Midwives
- Dentists
- Academics
- Government organizations
- NGOs

Nurse's Role in Community Collaboration

— The nurse:
- Maintains harmonious relationships
- Coordinates activities of health team members
- Promotes effective service delivery
- Helps achieve group goals

DEVELOPMENT OF TEAM AND COLLABORATION

Tuckman's Stages Of Team Development

1. FORMING

— Characteristics:
- Team is newly formed.
- Members depend on the leader.
- Members are learning about one another.
- Roles are not yet fully established.
- Leader may exercise strong control.

2. STORMING
— Characteristics:
- Resistance may occur.
- Conflict and tension develop.
- Members may challenge roles or leadership.
- Members may compete for influence.

3. NORMING
— Characteristics:
- Members develop cooperation.
- Roles become clearer.
- Members recognize each other's competence.
- Members begin functioning as colleagues.

4. PERFORMING
— Characteristics:
- Team is highly functional.
- Members demonstrate interdependence.
- Authority is exercised by competent members.
- Team focuses on accomplishing goals.

TOOLS FOR FACILITATING HEALTH CARE TEAMWORK

1. TASK ROLES
Purpose:
— Help the team accomplish its task.
Types:
1. Initiating/energizing
2. Information/opinion giving
3. Information/opinion seeking
4. Reality testing/clarifying
5. Coordinating
6. Orienting
7. Procedural technician

2. MAINTENANCE ROLES
Purpose:
— Help the team function effectively as a team.
Types:
1. Harmonizing
2. Gatekeeping
3. Encouraging
4. Following
5. Climatizing

3. INDIVIDUAL ROLES
Purpose:
— These behaviors attempt to satisfy individual rather than team goals.
Types:
1. Blocking/Aggressing
2. Out of field
3. Digressing
4. Recognition seeking

COLLABORATION MODELS

1. PRECEPTORSHIP MODEL
— Clinical teaching
— Orientation
— Skill development
— Feedback
— May evaluate learner

2. MENTORSHIP MODEL
— Professional development
— Career guidance
— Counseling
— Long-term development

3. LECTURER-PRACTITIONER MODEL
— Academic + clinical educator
— Facilitates clinical learning
— Supports assessment

Nurse's Role In Establishing Collaborative Relationships
A. Maintain Good Interpersonal Relationships
— The nurse maintains relationships:
- Intra-agency
- Inter-agency

B. Respect Other Team Members
— The nurse should:
- Recognize professional roles
- Respect expertise
- Avoid unnecessary competition
- Promote mutual understanding

C. Act As Liaison
— The nurse serves as a link between the patient and healthcare team.

D. Act As Patient Advocate
— The nurse represents the patient's:
- Needs
- Rights
- Preferences
- Concerns

— during interprofessional decision-making.

INTERPERSONAL RELATIONSHIPS IN TEAMWORK

Definition
— Reciprocal social and emotional interaction between two or more people who share common interests and goals.
Importance to Nursing
— Good interpersonal relationships:
1. Build functional multidisciplinary teams
2. Improve communication
3. Improve coordination
4. Improve cooperation
5. Build mutual understanding
6. Improve decision-making
7. Improve problem-solving

DYNAMICS OF INTERPERSONAL RELATIONSHIPS

1. DYAD
— 2 people
— Simplest form

— Direct communication between two people

2. TRIAD
— 3 people
— Third person can help mediate conflict

3. GROUP
— More than 3 people
— Collection of dyads and triads

TYPES OF RELATIONSHIPS AMONG HEALTH PROFESSIONALS

1. COMPLEMENTARY
— One person is dominant.
— One person is submissive.
— Control is unequal.

2. SYMMETRICAL
— Control is more evenly distributed.
— Both can express opinions.
— Power struggles may occur.

3. PARALLEL
— Control moves back and forth.
— Participants take turns holding control.
— Flexible communication.

Nurse–Physician Relationships

1. Collegial
— Equal trust
— Equal power
— Equal respect

2. Collaborative
— Mutual trust
— Mutual respect
— Shared power

3. Student–Teacher
— Either nurse or physician may teach

4. Friendly Stranger
— Formal information exchange
— Neutral emotional tone

5. Hostile/Adversarial
— Anger
— Verbal abuse
— Threats
— Conflict

PEPLAU'S INTERPERSONAL RELATIONSHIP PHASES

1. PRE-ORIENTATION
— Prepare
- Nurse's self-awareness
- Review patient information
- Plan interaction

2. ORIENTATION
— Meet and establish trust
- Initial encounter
- Clarify roles
- Identify problems
- Establish trust
- Identify needs

3. WORKING
— Solve the problem
— Identification
- Empathy
- Identify patient's problems
- Use clear communication
- Identify resources

— Exploitation
- Patient explores and uses available resources

4. TERMINATION
— End the professional relationship
- Patient's needs have been addressed
- Relationship appropriately ends
- Maintain healthy emotional boundaries

MIDTERMS: NCMB 419 (NLM) - WEEK #9.1: NURSING MANAGEMENT FUNCTIONS

ORGANIZATIONAL THEORY AND BUREAUCRACY

— **Max Weber** is known as the father of organizational theory. According to him, bureaucracy is an institutional method for applying general rules to specific cases, making the actions of management fair and predictable.

Characteristics of bureaucracy are the following:

— There must be a clear division of labor.
— A well-defined hierarchy of authority must exist which separates the superiors from subordinates.
— There must be impersonal rules and impersonality of interpersonal relationships.
— A system of procedures for dealings with work situations must exist.
— A system of rules covering the rights and duties of each position must be in place.
— Selection for employment and promotion is based on technical competence.

ORGANIZING

— is the process of establishing formal authority. It involves setting up the organizational structure through identification of groupings, roles and relationships, determining staffing patterns and distributing in the various areas as needed.

Elements Of Organizing

— The elements of organizing include setting up the organizational structure, staffing, scheduling, and developing job descriptions.

Setting Up The Organizational Structure

— The creation of an organizational system compatible with the philosophy, conceptual framework, and goals of the organization provides the means for the accomplishment of an organization's purpose.

— Understanding the organizational structure as a whole facilitates the development of roles and relationships to enable the achievement by goals.

Setting Up The Organization Structure May Achieve It's Purpose:

1. It informs members of their responsibilities so that they may carry them out
2. It allows members and the individual workers to concentrate on his/her specific role and responsibilities
3. It coordinates all organizational activities so there is minimal duplication of effort or conflicts.
4. It reduces the chances of doubt and confusion increasing assignments.
5. Avoids overlapping of functions because it pinpoints responsibilities.
6. Shows to whom and for whom they are responsible.

— Other individuals that the nurse may need to be accountable to include the immediate supervisor, the patient and family, central administration, and the physician because all of them participate in the nurse's work.

TYPES OF ORGANIZATION CLASSIFIED BY NATURE OF AUTHORITY

➢ **LINE ORGANIZATION**

— the simplest and most direct type of organization in which position has general authority over the lower positions in the hierarchy
— Line of authority flows from top to bottom

➢ **INFORMAL ORGANIZATION (FLAT/ HORIZONTAL)**

— refers to horizontal relationships rather than vertical.
— This is composed of small groups of workers with similar interest.

➢ **STAFF ORGANIZATION**

— is purely advisory to the structure with no authority to put recommendations into action.

➢ **ORGANIZATION**

— consists of the structure and process which allow the agency to enact its philosophy and utilize its conceptual framework to achieve its goals. It refers to a body or persons, methods, policies and procedures arranged in a systematic process through the delegation of function and responsibilities

➢ **FUNCTIONAL ORGANIZATION**

— is one where unit is responsible for a given part of the organization with workload.
— *Ex: All Standing and Ad Hoc Committees*

➢ **ORGANIZATIONAL CHART**

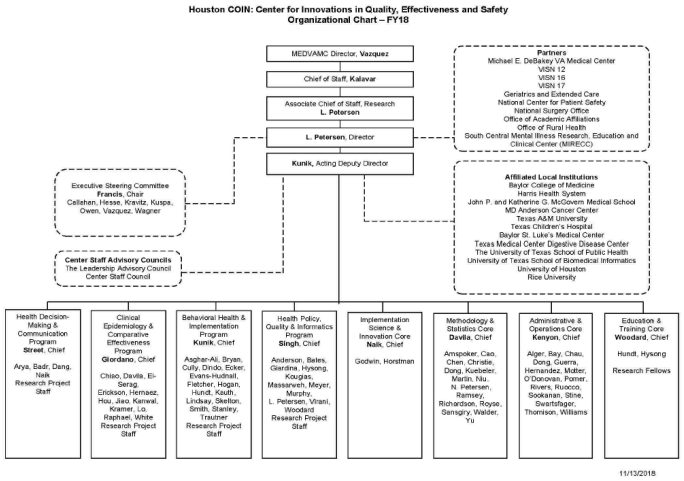
— is a line drawing that shows how the parts of an organization are linked. It depicts the formal organizational relationships, areas of responsibility, persons to whom one is accountable and channels of communications

There are two lines in the organizational chart:

a. Unbroken Solid lines are classified into two:
— The **solid horizontal lines** represent communication between people with similar sphere of responsibility and power but different functions.
— The **solid vertical lines** between positions denote the official chain of command and formal path of communication and authority

b. Dotted or broken lines

— represents staff position. Staff member provides information and assistance to the manager but has limited organizational authority.
— It also provides for specialization but does not have legitimate authority as it acts in an advisory capacity.



Characteristics Of An Organizational Chart

— **Division of Work** – box represents the individual or sub-unit responsible for a given task of the organizations work load

— **Chain of Command** – lines indicate who reports to whom and by what authority

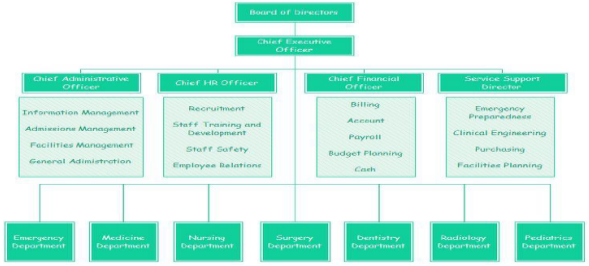
— **Type of Work to be Performed** – indicated by labels or descriptions for the boxes

— **Grouping of work Segments** – shown by the clusters of work groups (departments or single units).

— **Levels of Management** – indicate individual and entire management hierarchy.

Kinds Of Organizational Chart

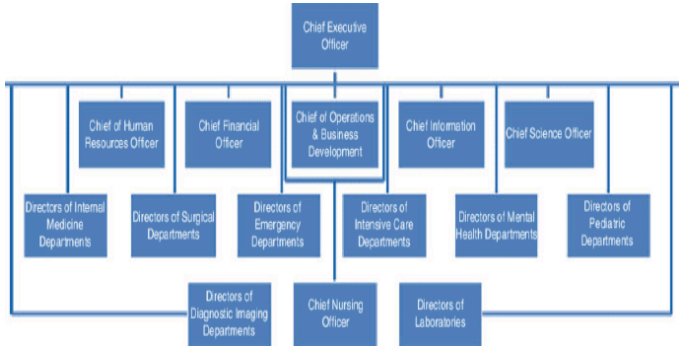
1. Structural Chart



— shows the various components of the organization and outlines the basic interrelationships.

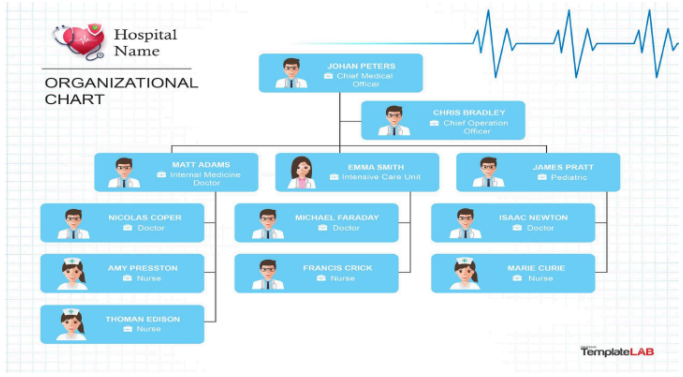
2. Functional Chart

— reflects the functions and duties of the components of the organization and indicates the interrelationships of these function.



3. Position Chart

— specifies the names, positions, and titles or ranks of the personnel.



ORGANIZING PRINCIPLES

1. UNITY OF COMMAND

— employee should be responsible to only one superior.

2. SCALAR PRINCIPLE/ HIERARCHY / CHAIN OF COMMAND

— authority and responsibility should flow in clear unbroken lines from highest executive to the lowest.

The following must be observed:

- when the job was given to a subordinate, he/she is responsible over resources needed to accomplish the task.
- though a particular function is being given to a subordinate, the responsibility of the superior is not being diminish.
- when the subordinate was given responsibility , he is accountable for his own actions to his/her superior.

3. HOMOGENOUS ASSIGNMENT OR DEPARTMENTALIZATION

— grouping together of employees with similar assignments.

4. SPAN OF CONTROL

— the number of workers that a supervisor can effectively manage should be limited,depending upon the pace and pattern of the working area.

5. EXCEPTION PRINCIPLE

— recurring decisions should be handled in a routine manner by lower level managers whereas problems involving unusual matters should be referred to the higher level.

6. DECENTRALIZATION OR PROPER DELEGATION OF AUTHORITY

— is the process of pushing decision making to the lower levels of the organization.

7. THE PRINCIPLE OF REQUISITE AUTHORITY

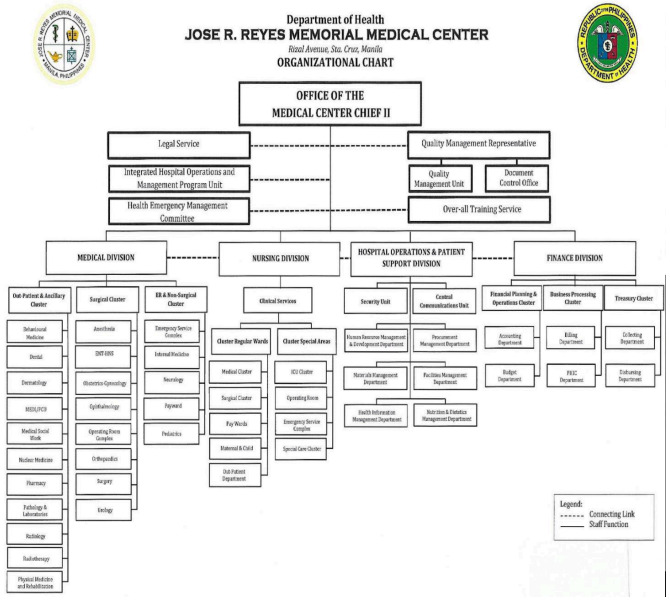
— When a particular task is delegated to a subordinate, the latter must also be given authority over resources needed for task accomplishment.

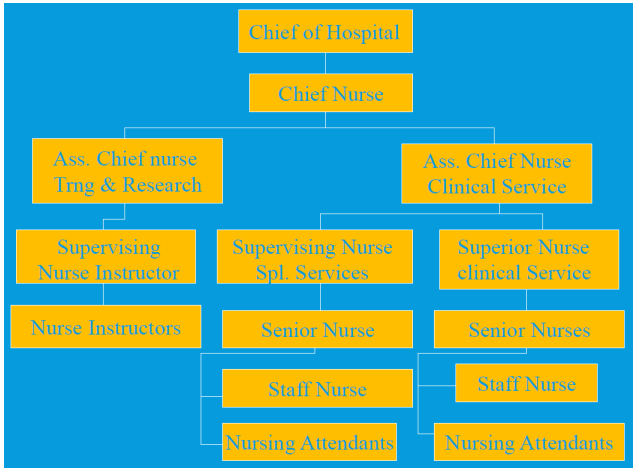
8. THE PRINCIPLE OF ORGANIZATIONAL CENTRALITY

— Nursing personnel interact with the greatest number of other healthcare workers, receive the greatest amount of work-related information and become most powerful in organizational structure.

9. THE PRINCIPLE OF ESPRIT D' CORPS

— This means teamwork and implies that in unity, there is strength.





ORGANIZATION OF THE NURSING SERVICE/DIVISION

— The nursing service as an organization constitutes the single largest group of hospital employees responsible of setting standards for safe nursing practice, providing quality care to the patients, and coordinating its services with the various divisions/departments/services in the hospital and community

— The Nursing service/division is administered by the Chief Nurse. under her.

In specialty hospital such as:

- Lung Center of the Philippines
- National Kidney Transplant Institute
- Philippines Children's Medical Center = the heads of the Nursing Department are called Department Managers. They still have a Chief Nurse under them.
- PGH -Asst. Director for nursing

➢ **The Chief Nurse**

— is directly responsible to the chief of the hospital or medical center chief.

— assisted by an assistant chief nurse.

➢ **Supervising Nurse**

— in charge of two or more nursing units.

➢ **Head Nurse Of Senior Nurse**

— responsible for the management of a nursing unit.

➢ **Staff Nurse**

— provides direct patient care and are assisted by nursing attendants

➢ **Nursing Attendants**

— performs simple, routine tasks for which they have been trained.

FORMS OF ORGANIZATIONAL STRUCTURE

1. TRADITIONAL HIERARCHICAL STRUCTURE (Tall, Centralized, Bureaucratic)

— This is commonly called line structure. Authority and responsibility are clearly defined leading to simplicity of relationships. This is associated with the principles of command, vertical control and coordination levels, and downward communications.

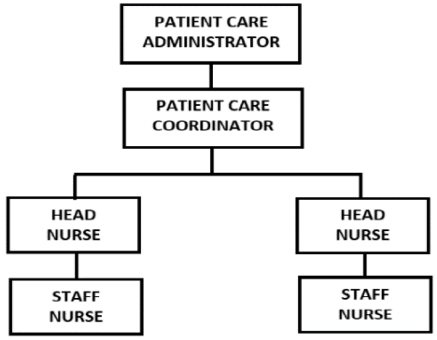
Merits Of Line Organization

1. Simplest
2. Unity of Command
3. Better discipline
4. Fixed responsibility
5. Flexibility
6. Prompt decision

Organizational Chart Of A Nursing Service Showing Several Layers Of Positions Between Chief Nurse And Nursing Staff.

Chief of Hospital → Chief Nurse → Supervising Nurse → Senior Nurse → Staff Nurse → Nursing Attendant

Organizational Chart Using The Titles Of Patient Care Administrator And Coordinator

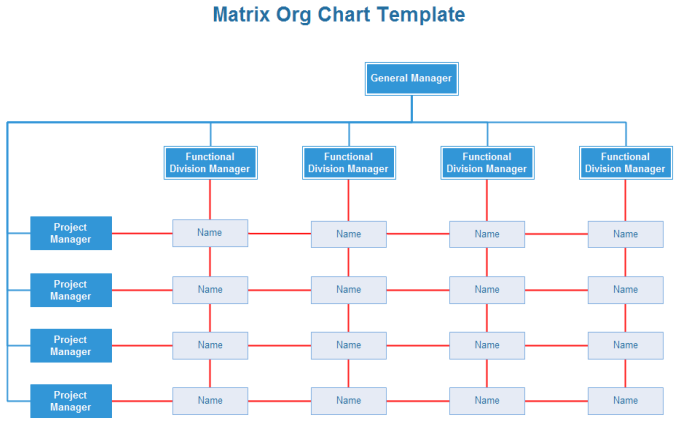


2. DECENTRALIZED (Flat, Horizontal, Participatory)

— The authority is shifted downwards to its divisions, services, and units. The decision making can occur where the work is being carried out, thereby professionals who do the job can participate in managing the organization.

3. MATRIX

— This is designed to focus on both the product and function. The manager of the unit responsible for a service reports both to a functional and product manager.
— A term applied to organizational structure that operate with characteristics of different types of structures.



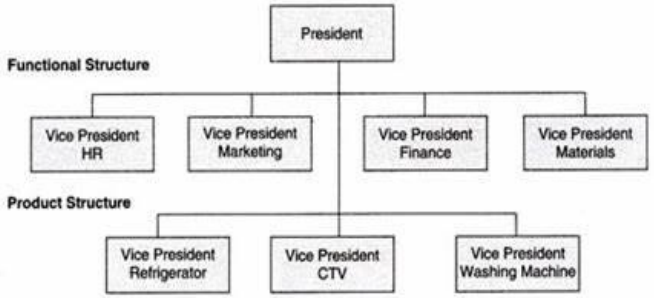


Figure 4.3 Hybrid organizational structure

MIDTERMS: NCMB 419 (NLM) - WEEK #9.2: THE MANAGEMENT PROCESS

PLANNING

— defined as pre determining course of action in order to arrive at a desired result.
— the most basic and essential activity of management functions that decide in advance what needs to be done for the day, month, or years ahead.
— defined as deciding in advance what to do; who is to do it; and how, when and where it is to be done.

➢ **HENRY FAYOL (1841-1925)**

— The first element of management defined by Henry Fayol is planning in which he defines as making a plan of action to provide foreseeable future.
— According to him that plan must have unity, continuity, flexibility, and precision. The plan should be included annual and 1o year forecasts, taking advantage of input of others. Planning improves experience, gives sequence in activity, and protects business against undesirable changes. Planning facilitates the art of handling people.
— **Planning** is designed for the nurse administrators who are seeking more effective means of improving current and future performance. It offers principles of planning and decision-making guides in nursing practice. It deals with those aspects of planning that directly affects the nurse administrators on a personal as well as organizational level.

Purpose Of Planning

1. Planning increase the chances of success.
2. It forces analytic thinking and evaluation of alternatives
3. It establishes a framework for decision making that is consistent with top Management objectives.
4. It orients people to action instead or reaction.
5. It includes day-to-day and future-focus managing.
6. It helps to crisis management and provides decision-making flexibility.
7. It provides a basis of for managing organizational and individual and individual Performance.
8. It increases employee involvement and improves communication.
9. It is cost effective.

Importance Of Planning

1. Leads to achievement of goals and objectives.
2. Gives meaning to work.
3. Provides for effective use of available resources and facilities.
4. Helps in coping with crisis.
5. Cost-effective
6. Based on facts and future activities
7. Leads to realization of the need for change
8. Planning improves communication and team collaboration through people involvement in planning activities.
9. Provides basis for control.
10. Necessary for effective control.

Principles Of Planning

1. Planning is always based & focused on the vision mission, philosophy, and clearly defined objectives of organization.
2. Planning is a continuous process
3. Planning should be pervasive.
4. Planning utilizes all available resources
5. Must be precise in its scope & nature
6. Should be time bound
7. Projected plan must be documented

Scope Of Planning

— **Top Management** – set the overall goals and policies of an organization
— **Middle Management** – direct the activities to actually implement the broad operating policies of the organization such as staffing delivery service to the units.
— **Lower or First level Management** – do the daily or weekly plans for the administration or direct patient care in their respective unit.

Major Aspects Of Planning

1. Planning should contribute to objectives.
2. Precedes all other processed of management.
3. Planning pervades all level
4. Planning should be efficient

Characteristics Of A Good Plan

— Have clearly worded objectives
— Guided by policies and procedures
— Indicate priorities
— Develop action that are flexible and realistic
— Develop logical sequence of activities
— Include practical methods for achieving objectives
— Pervade whole organization

Basis In Planning The Nursing Service

A. FORECASTING

— it helps managers look into the future, including the environment in which the plan will be executed.
— The hospital, this includes the type of hospital served (level 1, level 2, level 3, or level 4); the kind of service it offers (general or special) its philosophy mission and goals and categories of their budget (national or local).
— The community it serves, this includes the kind of people served their needs, expectations, literacy rate, economic levels, employment rates, demographic statistics, cultural values, values and services available in the community.
— The goals of care, vary according to the needs of the community, agency, trends in technology and in changing needs of the community, agency, trends in technology and in changing concepts of the nurses' roles and functions.

B. SETTING THE VISION, MISSION, PHILOSOPHY, GOALS AND OBJECTIVES

— **Mission** – statement outlines the agency's reason for existing, who the target clients are and the services will be provided.
— **Vision** – statement outlines the organization's future role and function. It gives the agency something to strive for.
— **Philosophy** – is a statement of beliefs and values that direct one's life or one's practice.
— **Goals** – are broad statements or objective that is expected to be achieved.
— **Objectives** – are the action commitments through which an organization's mission and purpose will be achieved and philosophy or belief sustained.

C. DEVELOPING AND SCHEDULING PROGRAMS

— Planning Formula
- Who, What, Where, When, Why, How

Modes Of Planning

1. Reactive planning – occurs after problem exist. Problems are dealt with separately w/o integration with the whole organization.
2. Inactivism – seek status quo and they spend their energy preventing change and maintaining conformity.
3. Preactivism – utilize energy to accelerate change and are future oriented.
4. Interactive or Proactive – consider the past, present and the future and attempt to plan the future of their organization rather than react to it.

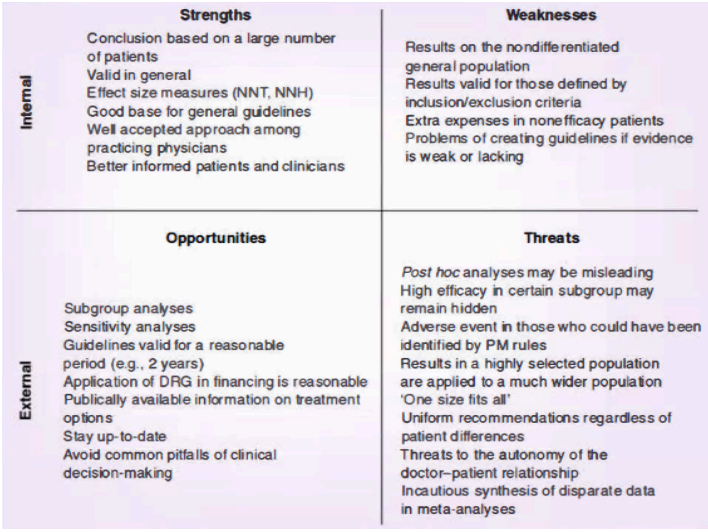
Types Of Planning

➢ **STRATEGIC PLANNING**

— Is a broad continuous systematic process that emphasizes assessment of the organizational environment both internally and externally such as economic, political, social, and technological factors.
— It is a management tool that helps organizations set long-term goals, a risk-taking decision with knowledge of their effects in the future, and evaluating the outcomes through reliable feedback mechanism.
— It focuses on performance improvement and utilizes strategies to accomplish the organization's desired outcomes
— Strategic planning forecasts the future success of an organization by matching and aligning an organization's capabilities with its external opportunities. An organization could develop a strategic plan for dealing with the nursing shortage, preparing succession managers in the organization, developing a marketing plan, redesigning workload, developing partnership, or simply planning for organizational success.

➢ **SWOT ANALYSIS**

— One effective tool that can assist in strategic planning is SWOT analysis (identification of strengths, weakness, opportunities, and threats) is one of the most commonly used in health care organizations. SWOT analysis is also known as TOWS analysis was developed by Albert Humphrey at Stanford University in the 1960s and 1970s.
— The first step in SWOT analysis is to define the desired end state or objective. After the desired objective defined, the SWOT are discovered and listed. Decision makers must then decide if the objective can be achieved in view of the SWOTs. If the decision is no, a different objective is selected and the process repeats. With the results of your analysis the following should be included: 1) working out what you hope to achieve, 2) being objective, 3) incorporating the findings into the action plan, and 40 revisiting your findings on a regular basis.
— Perform correctly, SWOT allows strategic planners to identify those issues most likely to impact a particular organization or situation in the future and then to develop an appropriate plan of action
- S – strength
- W – weakness
- O – opportunities
- T – threats



Strategic Planning As Management Process

1. Clearly define the purpose of the organization.
2. Establish realistic goals and objectives consistent with the mission of the organization.
3. Identify the organization's external constituencies or stakeholders and then determine their assessment of the organization's purposes and operations.
4. Clearly communicate the goals and objectives to the organization's constituents.
5. Develop a sense of ownership of the plan.
6. Develop strategies to achieve the goals.
7. Ensure that the most effective use is made of the organization's resources.
8. Provide a mechanism for informed change as needed.
9. Provide a mechanism for informed change as needed.
10. Build a consensus about where the organization is going.

5-Year Development Plan

Objectives	Strategies/ Activities	Target	Time Frame
Provision of adequate staffing	Adequate staffing	Nurse & Nurse Attendant Medical-50 Surgery 50 ER-20	As needed 2020-2025

Person/s Responsible	Resource	Indicator
Chief Nurse	Salary of Nurses & Nurse Attendant Benefits	No. of percentage age of Nurses and Nursing attendant hired

➢ OPERATIONAL PLANNING

— Is a detailed work plan or written blueprint in which the objectives of a nursing unit/department are put into measurable actions?

— It is known as a management plan.

— Some categorical areas for objectives are: patient satisfaction, patient safety, internal process, staffing, training and education, research, and financial.

— It includes the what (task to be undertaken), who (person responsible), when (timeline for task completion), and how much (amount of financial resource to carry out the task).

Operational Plan

Objectives	Target	Strategy	Activities
Provide excellent, reflective continuous patient-centered nursing care plan in collaboration with the clientele and health care team	Nursing service full support in the provision of better health outcomes	Continue with revision of Nursing manual	Schedule meeting of committee member Printing reproduction and distribution of manual

Time Frame	Budget	Person/s Responsible	Performance Indicator
Every Friday until end of September 2020 October 2020	10,5400 200 per copy for 32 wards	Chie Nurse Committee on Revision of Nursing Manual	100% approved NSO Manual approved and distributed

Difference between the strategic planning and operational planning.

Basic of Comparison	Strategic Planning	Operational Planning
Scope	General plan	Specific plan
Time Frame	3 – 5 years	1 year
Focus	Goal Setting process	Detailed |Information
Goal/Objective	Broad	Specific
Responsibility	Top management	Middle & and Line management
Approach	Downward	Upward

D. ESTABLISHING NURSING STANDARDS, POLICIES, AND PROCEDURES

— Nursing Standards Institutions develop their own standards of nursing practice, the standard of Nursing Practice and Nursing Service Administration formulated by the Association of the Nursing Service Administrators of the Philippines and revised 2008.

— The established standards in an evaluation provides professional desirable norms against which department's performance can be measured. Areas of improvement are identified and a plan of action to correct is made and implemented.

— Nursing Standards, the philosophy shall be based on the belief that the client is integral whole and that he is a unique individual with needs that can be met through nursing interventions.

— Nursing Service Policies are plans reduced to statements or instructions that direct organizations in their decision making. It direct individual behavior toward the organization's mission and define broad limits and desired outcomes of commonly recurring situations while leaving some discretion and initiative to those who must carry out that policy.

— Policies may also be implied or expressed. Implied policies neither written nor expressed verbally, have usually developed overtime and follow a precedent.

— While expressed policies are delineated verbally or inwriting. Most organizations have many written policies that are readily available to all people and promote consistency of action.

— Procedures are plans that establish customary or acceptable ways of accomplishing a specific task, and delineate a sequence of steps that required action. Procedures identify the process or steps needed to implement a policy and are generally found in manuals at the unit level of the organizations, (Marquis and Huston, 2017).

— Guidelines outlining the scope and standards of practice for specialty practice areas.

➢ KURT LEWIN (1951)

— In the mid-20th century Kurt Lewin (1951) developed change theory, identified 3 phases in which the change agent must proceed before a planned change becomes part of the system:

1. **Unfreezing** – this occurs when the change-agent convinces members of the group. To change or when guilt, anxiety, or concern can be elicited. Change agent needs to have made a thorough and accurate assessment of the extent of and interest in change, for them to become effective as change agent.
2. **Movement** – change agent must identifies, plans, and implements appropriate strategies, ensuring driving forces exceed restraining forces.

3. **Refreezing** – during this phase the change agent assists in stabilizing the system change so that it becomes integrated into the status quo. For this to occur, the change agent must be supportive and reinforce the individual adaptive behavior efforts of those affected by the change (Marquis and Houston, 2017).

BUDGET

— Annual operating plan
— Financial "road map" and plan
— Estimate of future costs
— Plan for utilization of capital in the operating programs
— A financial plan that includes estimated expenses as well as income for a period of time,

NURSING BUDGET

— is a plan for allocation of resources based on preconceived needs for a proposed series of programs to deliver patient care during one fiscal year.

HOSPITAL BUDGET

— is a financial plan to meet future service expectations. Budgeting translate these needs to manpower, equipment and supplies so that both services are provided at the highest level of quality at a minimum cost.

Types Of Budget

➢ **PERSONNEL BUDGET**

— it is the type of budget affected by personnel policies such as salary related to position and number of days allowed for educational and personal leave.

➢ **OPERATING OR REVENUE AND EXPENSE BUDGETS**

— this includes the daily revenues and expenses to operate the health care institution
— it deals primarily with salaries, supplies, and contractual services.

➢ **CAPITAL EXPENDITURE BUDGETS**

— related to long range planning
— include physical changes such as replacement, or expansion of the plant, major equipment and inventories

➢ **CASH BUDGET**

— plan to make adequate funds available as needed and to use an extra fund profitably.

➢ **FLEXIBLE BUDGET**

—— budgets that adjust automatically over the course of the year depending on variables such as volume of labor costs and capital expenditures.

Purpose Of Budgeting

1. A forecast of income and expenditure
2. A tool for decision making, to provide financial framework
3. Measured the financial report and its statistical data.

Importance Of Budget

— To control the over spending of the Nursing services.
— Budgeting helps you control the spending beyond the means in related to nursing activities.
— To monitor income and expenses of the Nursing services. Budgeting helps you keep on track of the income and expenditures of the Nursing services.
— Rationalization of the financial transparency and accountability of the Nursing services. Budgeting gives you a precise report summary of the financial responsibilities.

Budgeting Process

➢ **Phase 1: Gathering Data**

— Study the overall past performance based on proposed goal.
— Gather environmental information Regulates the overall expenses and revenues from the past reports to present based on proposed budget.

➢ **Phase 2: Planning**

1. Create a Budget Committee
2. Set Objectives based on strategic goals
3. Establish a program or project for future goals
4. Formulate a budget guideline to synchronize with financial linkages
5. Settle an Operating and Capital Budget
➢ 6. Manage a budget hearing
7. Prioritize your budget based on necessity
8. Conclude your proposed budget plan

➢ **Phase 3: Approval and Execution**

1. Summarized the proposed nursing service budget plan and let the authorize finance budget committee to review and approve the budget.
2. Make a budget plan ready for improvement in order to calibrate any errands to the General Appropriations Act (GGA).
3. Cascade the proposed budget plan accordingly to all nursing unit staff and department.

➢ **Phase 4: Monitoring and Reporting**

1. Observe the proper delivery or implementation of the budget plan.
2. Make an analysis between the proposed budget and the actual performance.
3. Clarify and justify the results to determine the significant trends
4. Ready to make an adjustment based on the required needs.
5. Present the financial report according to hospital policy.

Pre-Requisites For Budgeting

1. Sound organizational structure with clear lines of authority and responsibility is needed.
2. Non monetary statistical data – such as number of admission, average length of stay, percentage of occupancy and number of patient days – are used for planning and control of the budgetary process.

3. Chart of accounts are designed to be consistent with the organizational plan.
4. Managerial support is essential for a budgetary program.
5. Formal budgeting policies and procedures should be available in the budget manual.

Factors In Budget Planning

1. Type of patient and the length of stay in the hospital.
2. Bed capacity and hospital size
3. Physical plant of the hospital, design and the size of the wards/unit and other treatment rooms.
4. Personnel policies.
5. Grouping of patients
6. Standard of Nursing Care
7. Method of performing nursing care
8. Method of documentation
9. Proportion of nursing care providers (professional / non-professional)
10. Amount and quality of supervision available and provided
11. Competencies of job description and qualification
12. Method of patient assignment
13. Amount and kind of labor-saving devices and equipment
14. Amount of centralized service provided
15. Nursing service requirement ancillary departments
16. Reports required by administration
17. Affiliation of nursing students or medical students

Budgeting Methods

— **Incremental Budgeting** – simplest method for budgeting. Since the budget for the next coming year may be projected, the programs and services were not prioritized.
— **Zero-Based Budgeting** – the method does not automatically claim to be funded. A set funding priority is the main purpose of this method.
— **Flexible Budgeting** – this method calculates what the expenses should be specified in the program
— **Performance Budgeting** – emphasizes the outcomes and results instead of activities and outputs.

Factors In Determining Budgetary Requirement

— Asses the appropriate provision in the current General Appropriation Act.
— Associate the sources of funds (General, national, city, municipal…)
— Evaluate the current appropriations and actual expenditures for the current year.
— Analyze the projected changes in other department that will affect the nursing service budget.
— Consider the required expenditures, supplies, equipment and material, repair and replacement for the next coming year.
— Projects the estimated personnel salaries and benefits including their unusual leaves.
— Assess the cost of Human Resource Development and Research Programs.
— Render this information into peso and submit the official forms to the Medical Center Chief for approval and inclusion in the general budget.

TIME MANAGEMENT

— It is a technique for allocating one's time through the setting of goals, assigning priorities, and identifying and eleminating wasted time, and using managerial techniques to reach goals efficiently.
— …..One of the strategies in managing the time is learn to prioritize duties, managing and controlling crisis, reducing stress and balancing work and personal time (Marquis and Houston,2017).
— …nothing is particularly hard if you divide it into small jobs. – Henry Ford
— …things which matter most must never be at the mercy of things that matter least. – Johann Wolfgang von Goethe

Time Management Principles

a. Planning anticipates the problem that arise from actions without thought.
b. Tasks to be accomplished should be done in sequence and should be prioritized according to importance.
c. Setting deadlines in one's work and adhering to them is an excellent exercise in self-discipline.
d. Deferring, postponing, or putting off decisions, actions, or activities can become a habit which oftentimes causes lost opportunities and productivity, generating personal or interpersonal crises.
e. Delegation permits a manager to take priority for decision making and to assign tasks to the lowest possible consistent with his/her judgement, facts, and experience, (Venson, 2016).

Time-Saving Techniques, Devices, And Methods To Better Use Of Time

1. Conduct an inventory of your activities
2. Set goals and objectives and write them down.
3. With the use of calendars, executive planners, logs or journals, write what you expect to accomplish yearly, monthly, weekly or daily.
4. Break down large projects into smaller parts.
5. Devote a few minutes to concentrate on one thing at a time.
6. Organize your work space so it is functional.
7. Close your door when you need to concentrate.
8. Learn to delegate.
9. In a meeting, define the purpose clearly before starting.
10. Take or return phone calls during specified time. Develop effective decision-making skills. Do no afraid to say "no".
11. Take rest breaks and make good use of your spare time

Three Steps to Time Management

a. Allow time to planning and establish priorities.

b. Complete the highest task whenever possible and finish one task before beginning another.

c. Reprioritize based on the remaining tasks and on new information that may have been received.

MIDTERMS: NCMB 419 (NLM) - WEEK #10: STAFFING

STAFFING

— process of determining and providing the acceptable number and mix of nursing personnel to produce a desired level of care to meet patient's demand.

FACTORS AFFECTING STAFFING

1.Patient Factors

— Census fluctuations
— Patient acuity
— Level of care/degree of dependence
— Patient age group
— Special treatment and procedure
— Communicability
— Rehabilitation
— Patient and family care demands/expectations

2. Staff Factors

— Number of nursing staff available
— Ratio of professional to non-professional
— Number of leaves
— Turn-over rate
— Span of supervision

3. Nursing service factors

— Nursing care modality in use
— Type of services/patient classification system
— Patterns of work schedule
— Training and staff development programs
— Research activities

4. Health care organization factors

— Type of hospital
— Services offered/population served
— Work time policy
— Administrative policy on weekend and holiday duty
— Presence of support services
— Nursing unit architectural design
— Availability of resources like equipment, materials and supplies
— Technology anticipated
— Projected units of services
— Budget limitations

PATIENT CARE CLASSIFICATION SYSTEM

— Is primary developed to determine workloads requirements and staffing needs. It is categorizing patients on the basis of certain needs that can be clinically observe by the nurse. It is a method of grouping patients according to the amount and complexity of their nursing care requirements and nursing time and skill they require.

Nursing Care Hours Per Patient Per Day According To Classification Of Patients By Units

Cases/Patients	Nch/Patient/Day	Prof.To Non Prof. Ratio
1. General Medicine	3.5	60:40
2. Medical	3.4	60:40
3. Surgical	3.4	60:40
4. Obstetrics	3.0	60:40
5. Pediatrics	4.6	70:30
6. Pathologic Nursery	2.8	55:45
7. ER/ICU/RR	6.0	70:30

PATIENT CARE CLASSIFICATION SYSTEM

Level I

— Patients that can do self care or minimal care
— Non-emergency
— Requires little treatment
— NCH 1.5 hours/day
— P:NP Ratio: 55:45

Level II

— Moderate or intermediate care
— Requires periodic assessment and treatment
— NCH is 3hours/day
— P:NP ratio: 60:40

Level III

— Total, Complete or Intensive Care
— Completely dependent on the personnel for care
— NCH is 6hours/day
— P:NP ratio: 65:35

Level IV

— Highly specialized Critical Care
— Requires the maximum nursing care
— Continuous assessment, treatment and evaluation
— NCH 6-9hours/day
— P:NP ratio is 70:30 to 80:20

➢ **REPUBLIC ACT 5901 – Forty Hour Week Law**

- Hospital employees working in ≥100 bed capacity will work 40 hrs/week
- If it is < 100 bed capacity 48 hrs/week

Number of working and non-working days and hours of nursing personnel per year

Rights and Privileges Given Each Personnel per Year	Working Hours Per Week	
1. Vacation leave	15	15
2. Sick leave	15	15
3. Legal Holidays	10	10
4. Special Holidays	2	2
5. Special Privileges	3	3
6. Off— Duties as per R.A 5901	104	52
7. Continuing Education Program	3	3
Total Non-Working Days Per Year	152	100
Total Working Days per Year	213	265
Total Working Hours Per Year	1.704	2.120

RELIEVERS NEEDS

— *Compute for relievers needed, the following should be considered:*

1. Average numbers of leaves taken each year	15
a. Vacation Leave	10
b. Sick Leave	5
2. Holidays	12
3. Special Privileges as per CSC MC #6s. 1996	3
4. Continuing Education Program for	
a. Professionals	3
b. Total Averages Leaves	33

— To determine the relievers needed, divide 33 (the average number of days an employee is absent per year) by the number of working days per year that each employee serves (whether 213 or 265).

— This will be 0.15 per person who works 40 hours per week and 0.12 per person for those working 48 hours per week.

— Multiply the computed reliever per person by the computed number of nursing personnel. This will give the total number of relievers needed.

- Ave. Absent Per / Year Total Duty Days ∗ No. Of Nurses = No. Of Reliever
- If 40 hrs per week equivalent to 213 days per year
- If 38 hrs per week equivalent to 265 days per year

— Example:

- Total Nurses = 40
- Hrs Per Week = 48
- Total Absent Days Per Week = 33 (Constant)
- 33 / 265 * 40 = 4.28
- Say: 5 Relivers

DISTRIBUTION BY SHIFTS

— Morning shift (6-2)
— Needs 45-51% of nursing personnel
— Afternoon shift (2-10)
— 34-37%
— Night Shift (10-6)
— 15-18%

STAFFING COMPUTATION FORMULA

— To compute for the staff needed in the In-Patient unit of the hospital the following steps are considered:

1. Categorize the number of the patients according to the levels of care needed. Multiply the total number of patients according to the level of percentage of patients at each level of care. (whether minimal, intermediate, intensive or highly specialized)

Percentage Of Patients In Various Levels Of Care

Types Of Hospital	Minimal Care	Moderate Care	Intensive Care	Highly Specialized Care
Primary	70	25	5	–
Secondary	65	30	5	–
Tertiary	30	45	15	10
Special Tertiary	10	25	45	20

1. Categorize the Patients According to levels of care needed.

— *Ex: Personnel needs for 250 patients in a tertiary hospital*

- 250(pts) x .30 = 75 patients needing minimal care
- 250(pts) x .45 = 112.5 patients needing moderate care
- 250(pts) x .15 = 37.5 patients need intensive care
- 250(pts) x .1 = 25 patients need highly specialized nursing care

2. Find the total number of Nursing care hours needed by the patients at each category level.

a. Find the number of patients at each level by the average number of nursing care hours needed per day
b. Get the sum of the nursing care hours needed at the various levels.

Categories or levels of care of patients, nursing care hours needed for patient per day and ratio of professionals to non-professionals

Levels of Care	NCH Needed Per Pt. Per Day	Ratio of Pro. To Non. Prof
Level I: **Self Care of Minimal Care**	1.50	55:45
Level II: **Moderate or Intermediate care**	3.0	60:40
Level III: **Total or Intensive Care**	4.5	65:35
Level IV: **Highly Specialized or Critical Care**	6.0 7 or higher	70:30 80:20

- 75 pts x 1.5 (NCH needed at level I) = 112.5 NCH/day
- 112.5 pts x 3 (NCH needed at level II) = 337.5 NCH/day
- 37.5 pts x 4.5 (NCH needed at level III) = 168.75 NCH/day
- 25 pts x 6 (NCH needed at level IV) = 150 NCH/day
- Total = 768.75NCH/day

3. Find the total NCH needed by 250 patients per year

- 768.75 x 365 (days/year) = 280,593.75 NCH /year

4. Find the actual working hours rendered by each nursing personnel per year

- 8 (hours/day) x 213 (working days/year) = 1,704 (working hours /year)

5. Find the total number of nursing personnel needed.

a. Total NCH per year = 280,593.75 / 1704 = 165 working hrs/year
b. Relief x total Nursing Personnel = 165 x .15 = 25
c. Total Nursing Personnel needed 165 + 25 = 190

6. Categorize according to professional and non-professional personnel. Ratio professional to non- professional in a tertiary hospital is 65:35

- 190 x .65 = 124 professional nurses
- 190 x .35 = 66 nursing attendants

7. Distribute by shifts

- 124 Nurses x. 45 = 56 nurses on AM shift
- 124 nurses x .37 = 46 nurses on PM shift
- 124 nurses x .18 = 22 nurses on night shift
- Total = 124 nurses

- 66 Nsg attendant x .45 = 30 Nsg attendants on AM shift
- 66 Nsg attendants x .37 = 24 Nsg attendants on PM shift
- 66 Nsg attendant x .18 = 12 Nsg attendants on night shift

SCHEDULING

— A schedule is a timetable showing planned work days and shifts for nursing personnel.
— The objective in scheduling is to assign working days and days-off to the nursing personnel so that adequate patient care is assured.

Factors Considered In Making Schedules

— Different levels of the nursing staff: adequate coverage for 24 hours, seven days a week:
— Staggered vacations and holidays: weekends:
— Long stretches of consecutive working days: evening and night shifts: and floating.
— An adequate mix of nurses and nursing attendants education, training and experiences.
— Afternoon and night shift requirements for the staff are usually lower than in the morning shift.
— Lower staff requirements on Saturdays and Sundays since there are lesser medical rounds, fewer medical orders and lower patient census.
— Schedules for holidays are staggered at least once a month. Vacations, whether forced or requested,
— Long stretched of consecutive working days
— Busy units may require additional help.
— Unscheduled absences may require a staff to be pulled out from her regular area
— Job dissatisfaction and high turnovers rates.

ASSESSING A SCHEDULING SYSTEM

— Scheduling may vary from agency to agency, but the scheduling system must function smoothly in terms of:
1. **Ability** – to cover the needs of the unit a minimum required number of staff must meet the nursing needs of the patients in the units at all shifts
2. **Quality** – to enhance the nursing personnel's knowledge, training and experience While permanent assignment to one unit enhances skills in caring for a particular kind of patient (whether obstetrical, medical, surgical or pediatrics), many nurses who have future plans of going into teaching, or specialization or even working abroad, would prefer to experience being assigned to various units before settling down to a particular unit of their choice
3. **Fairness** – to the staff All nursing personnel should get a fair share of weekends, holidays offs, rotation patterns for the whole year including assignment to "difficult" or "light" or "undesirable" units or shifts
4. **Stability** – The nursing personnel would like to know in advance their schedule of assignment so that their personal schedules (whether at home, social and civic responsibilities are in harmony with each other: and
5. **Flexibility** – the ability to handle changes brought about by emergency leaves, scheduled or unscheduled leaves of absences.

TYPES OF SCHEDULING

1. **Centralized Schedule** – One person, usually the Chief Nurse or her designate, assigned the nursing personnel to the various units of the hospital. This includes the shifts on duty and off-duty.
2. **Decentralized Schedule** – shift and off-duties are arranged by the Supervising Nurse or Head or Senior Nurse of the particular unit.
3. **Cyclical Schedule** – covers a designated number of weeks called the cycle length and is repeated thereon. It assigns the required number of nursing personnel to each nursing unit consistent with the unit's patient care requirements, the staff's preference, their education, training and experience.

The following scheduling variables should be considered:

a. length of scheduling period whether 2 or 4 weeks;
b. shift rotation;
c. week-ends off;
d. holidays offs;
e. vacation leave;
f. special days (birthdays, wedding anniversary, etc);
g. scheduled events in the hospital, training programs, or meetings;
h. job categories;
i. tardiness and;
j. continuing professional education (CPE) programs.

Advantages Of Cyclical Schedule

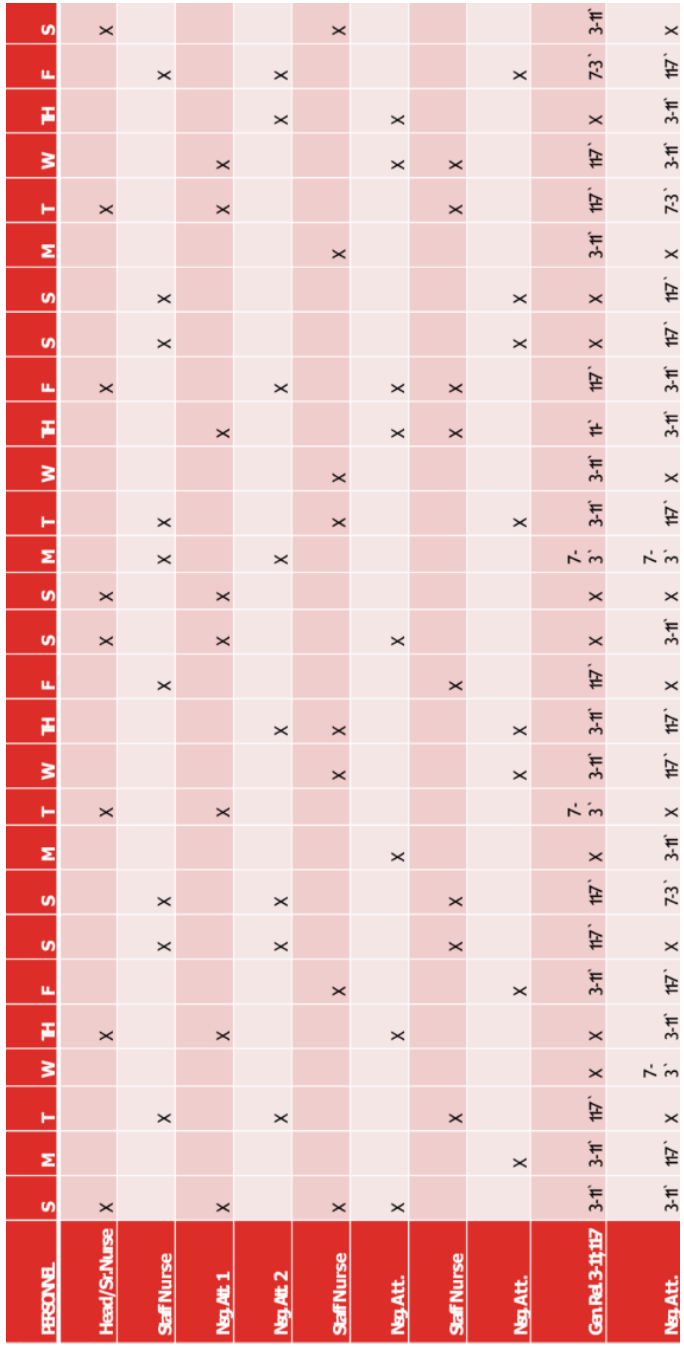
— It is fair to all. Favoritism is minimized as all nursing personnel get their fair share of rotation to the various shifts.

— It saves time as the schedule does not have to be redone every week or two.

— It enables the employees to plan ahead for their personal needs preventing frequent changes in schedule.

— Scheduled leave coverage such as vacation, holidays and sick leaves are more stable.

— Productivity is improved.



MIDTERMS: NCMB 419 (NLM) - WEEK #11: APPLICATION OF RESEARCH IN NURSING LEADERSHIP AND MANAGEMENT

RESEARCH

— helps nurses determine effective best practices and improve patient care.
— helps nursing respond to changes and challenges in the healthcare environment, individual, family, patient, and group populations, and government regulations.

Nursing research is

— systematic inquiry designed to develop knowledge about issues of importance to nurses, including nursing practice, nursing education, and nursing administration. Polit and Beck (2006).
— In this complex world of the 21st century, the role of the nurse has continuously evolved significantly.
— Though each role carries different responsibilities, the primary goal of a professional nurse remains the same: to be the client's advocate and provide optimal care on the basis of evidence obtained through research.

PATIENT SATISFACTION

— is an important and commonly used indicator for measuring the quality in health care.
— affects clinical outcomes, patient retention, and medical malpractice claims.
— It affects the timely, efficient, and patient-centered delivery of quality health care.

Research helps improve client satisfaction.

— Researchers demonstrated that there are differences in satisfaction levels among different genders.
— To explore further the reasons for these differences, a research made it separated to compare the specific causal links between the two gender samples and to draw broader inference from the results. Some findings provide support for the application of CS study to derive customer knowledge, which also reveals some interesting suggestions for future research.

ACTION RESEARCH

— In the health Care settings, the leadership, interpersonal, and management skills needed to manage such change effectively within multi-professional in a variety of health care settings. It provides unique use of action research as a model for planning and implementing change at the patient-service interface.

Example of Action Research that can be applied with Nursing Leadership and Management.

— Work-based learning: a leadership development example from an action research study of shared governance implementation
— Tracey Williamson L
- Affiliations expand, PMID: 16238690 DOI: 10.1111/j.1365-2934.2005.00576.x

— **Abstract**
- Aim: An empowering action research study was undertaken to evaluate and strengthen the implementation of shared governance. One aim was to identify factors that acted as aids or barriers to effective decision-making by clinical leaders. As a work-based learning approach, action research was expected to lead to integration of learning into practice by researcher and participants alike.

— **Background**
- Shared governance replaces traditional hierarchies and requires and develops clinical leaders. Strategies are needed to maximize learning from introduction of such initiatives at the individual, group and organizational level.

— **Methods**
- Participant observations and interviews were undertaken with shared governance council members from one model in north-west England.

— **Results**
- Leadership skills and knowledge and shared governance practices were significantly enhanced. Preparation for council roles was considered inadequate. Increased structured time for reflection and action planning was indicated.

— **Conclusions**
- Implementation of shared governance has succeeded in developing leadership capacity. Evaluation findings have led to improvements in the overall shared governance model. Action research has been found to have great utility at optimizing work-based learning. Nurse Managers need to develop their coaching and facilitating skills and recognize there is no "quick fix" for developing clinical leaders. Implications include the need to support learners in identifying and implementing changes arising from work-based learning activities, the significant resource implications and the need to optimize the organizational climate if work-based learning approaches to leadership and management development are to succeed.

FEASIBILITY

— can provide such data, and this can validate study procedures or identify procedures in need of revision.

Step-by-Step Guide to Feasibility Research

1. Identify a problem and/or a question.
2. Review the literature.
3. Identify gaps in our knowledge.
4. Refine the general question, formulating a specific research question(s).
5. Consider your reasons for conducting preliminary research and determine the form it should take.

a. If you want to evaluate the feasibility of carrying out the planned protocols and interventions of an anticipated larger study with randomization of participants, conduct a randomized pilot study.
b. If you want to evaluate the feasibility of using all or part of an intervention-and, possibly, other processes-in a proposed larger study, but without randomizing participants, conduct a nonrandomized pilot study.
c. If you want to evaluate aspects of data collection, data management, the adequacy of resources to carry out a study, or other processes to be undertaken in an anticipated future trial (excluding the specific intervention and exact protocol) with a small sample, conduct a feasibility study that is not a pilot study.

6. Design the study.
   a. Choose a research design (cross-sectional, cohort, or correlation, for example).
   b. Determine setting, sample size, recruitment strategy, randomization (if appropriate), instruments, data analysis, and procedures.
   c. Ensure protection of human subjects (submit plan for institutional review board approval).
7. Collaborate with stakeholders at the setting to minimize disruptions and obtain support.
8. Carry out the study.
9. Analyze the results.
10. Relate the findings to plans for a future study.
    a. Do results suggest it is worthwhile to pursue the study as planned?
    b. Do results provide data suggesting it would be important to modify aspects of this study to improve the anticipated larger study?
    c. Do results provide the data needed to propose a larger study as planned?
11. Disseminate your findings.

JOURNAL SHARING AND REACTION ON SHARED GOVERNANCE

➢ **REFLECTIVE JOURNALING**

— Is a term that refers to documenting your ideas, personal thoughts, and experiences through writing.
— A valued tool for teaching nursing students and for documentation, support, and generation of nursing knowledge among experienced nurses.
— A comfortable medium for nurses to be more open about their journey
and experience.
— Can also be a great way to map a nurse's progress and achievements in his or her nursing/clinical education as well as in their career.

SHARED RESPONSIBILITY FOR IMPLEMENTING EVIDENCE-BASED PRACTICE

— **Evidence-based practice (EBP)** is the process of collecting, processing, and implementing research findings to improve clinical practice, the work environment, or patient outcomes.
— According to the American Nurses Association (ANA), nursing interventions should be practical, methodical decisions based on EBP research studies. Utilizing the EBP approach to nursing practice helps us provide the highest quality and most cost-efficient patient care possible.

When evaluating EBP nursing research studies, focus on these four criteria:

— Validity
— Reliability
— Relevance
— Outcome
— Facilitating EBP is a shared responsibility of the professional nurse, the organization, leader-managers and the education or staff development department.

TRAINING NEEDS ANALYSIS

— is a process in which the company identifies training and development needs of its employees so that they can do their job effectively. It involves a complete analysis of training needs required of various levels of the organization.
— It is a process which helps the organization review the state of their training.
— Its importance is to helps them to determine all the training needs to be completed in a certain period to allow their team to complete their job as effectively as possible.
— The purpose of training needs analysis is to identify performance requirements and the knowledge, skills, and abilities needed by an agency's workforce to achieve the requirements. An effective training needs assessment will help direct resources to areas of greatest demand.

Three Levels of Training Needs Analysis

1. **Organizational Analysis** – TNA at the organizational level. *Ex: SWOT Analysis*
2. **Task Analysis** – Tasks to be performed on the job are analyzed; Knowledge, Skills, and Abilities needed to do those tasks are also analyzed
3. **Individual Analysis** – Who needs to be trained and what training is needed

Three key steps involved in training needs analysis to ensure your business is making the most of the process:

— Decide On Skill Sets
— Evaluate The Skills Of Staff
— Highlight The Skills Gap

Key Concerns of Upper and Mid-Level Managers and Trainers in Needs Assessment

	Upper-Level Managers	**Mid-Level Managers**	**Trainers**
Organizational Analysis	– Is training important to achieve our business objectives? – How does training support our business strategy?	– Do I want to spend money on training? – How much?	– Do I have the budget to buy training services? – Will managers support training?
Person Analysis	– What functions or business units need training?	– Who should be trained? – Managers? – Professionals? – Core employees?	– How will I identify which employees need training?
Task Analysis	– Does the company have the people with knowledge, skills, and ability needed to compete in the marketplace?	– For what jobs can training make the biggest difference in product quality or customer service?	– What tasks should be trained? – What knowledge, skills, ability, or other characteristics are necessary?